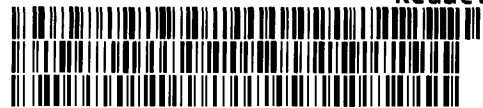


UID: A0DC18D5-245E-48D2-9BE9-9EABD30C19C5
TCGA-FY-A3NN-01A-PR

Redacted

Report for [REDACTED]



TEST: Surgical Pathology
Collected Date & Time:

Result Name Results Units Reference Range
Surgical Pathology SURGICAL P
SURGICAL PATHOLOGY REPORT

Patient Name: [REDACTED]
Accession #
Med. Rec. #:
Billing Type: Outpatient
Location:
DOB: (Age:
Service: Surgery
Gender: F
Billing
Taken Received Reported:
Physician(s): [REDACTED]

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed by:	Date Reviewed: 12/30/11	

Specimen(s) Received

A: Right medial ankle lesion
B: Total thyroidectomy
C: Right paratracheal level 6 lymph nodes

ICD-0-3

carcinoma, papillary, follicular variant
8340/3

Site: thyroid, NOS C13.9

Pathologic Diagnosis

- A. Skin (right medial ankle lesion), excision:
- Benign skin.
 - Thrombosed dermal vessel.
- B. Right and left thyroid, total thyroidectomy:
- Papillary thyroid carcinoma, right superior pole.
 - Tumor size: 2.0 cm.
 - No extrathyroidal extension identified.
 - Resection margins: Negative.
 - Adenomatous nodule, left thyroid, 1.8 cm.
 - 2 benign lymph nodes identified.

CAP CANCER CHECKLIST: THYROID, TOTAL THYROIDECTOMY

Procedure: Total thyroidectomy
Specimen Integrity: Intact

Specimen Size:

Right lobe: 5 x 4 x 2 cm
Left lobe: 5.4 x 2.8 x 1.8 cm
Isthmus: 1.7 x 1.5 x 0.3 cm

Specimen Weight: 34 g
Tumor Focality: Unifocal

Dominant Tumor:

Tumor Laterality: Right
Tumor Size: 2.0 cm

Histologic Type: Papillary carcinoma
Variant: Follicular
Architecture: Follicular
Cytomorphology: Classical
Margins: Uninvolved by carcinoma
Tumor Capsule: None
Lymph-Vascular Invasion: Not identified
Extrathyroidal Extension: Not identified

hw
1/7/12

PATHOLOGIC STAGE: pT1b, pN1a, pMX

NOTE: Information on pathology stage and the operative procedure is transmitted to this Institution's Cancer Registry as required for accreditation by the Commission on Cancer. Pathology stage is based solely upon the current tissue specimen being evaluated and does not incorporate other relevant data. Pathology stage is only a component to be considered in determining the clinical stage and should not be confused with nor substituted for it. The exact operative procedure is available in the surgeon's operative report.

Ancillary Studies: None performed

C. Right paratracheal level 6 lymph nodes:

- 2 lymph nodes, both positive for metastatic papillary thyroid carcinoma.

- Associated benign thymic tissue identified.

Primary Pathologist: [REDACTED]

Electronically Signed Out by: [REDACTED]

Clinical History

Median right ankle lesion. Papillary thyroid cancer.

Gross Description

A. Received in formalin labeled "medial right ankle lesion" is an ellipse of skin marked with a single suture at one tip and a double suture in the middle of one broad edge. The requisition form states "single stitch - anterior margin, double stitch - superior margin." With the double stitch designated as 12 o'clock, the single stitch is in the 9 o'clock position. According to this orientation, the specimen is 2.2 cm from 3 o'clock to 9 o'clock, 1.0 cm from 12 o'clock to 6 o'clock, and is 0.2 cm thick.

There is a raised, blue-gray lesion measuring 0.7 x 0.4 cm, at least 0.2 cm from the nearest skin margin at 5 o'clock. The margins are inked as follows: 12 o'clock half blue, 6 o'clock half yellow, 3 o'clock tip orange, 9 o'clock tip green. The specimen is serially sectioned from 3 o'clock to 9 o'clock. The specimen is entirely submitted with the 3 o'clock half in cassette "A1" and the 9 o'clock half in cassette "A2."

B. Received fresh labeled "total thyroid" is a total thyroidectomy specimen weighing 34 grams. The right lobe is 5.0 x 4.0 x 2.0 cm, and the left lobe is 5.4 x 2.8 x 1.8 cm, and the isthmus is 1.7 x 1.5 x 0.3 cm.

The right lobe is inked blue, the left lobe is inked black, and the isthmus is inked yellow.

Sectioning demonstrates a suspicious tan nodule in the right superior pole measuring 2.0 cm in greatest dimension. The nodule is encapsulated, is mostly red-brown, with a central area of firm, tan-gray, suspicious tissue. In the mid portion to inferior right lobe, there is a well-circumscribed, diffusely hemorrhagic, pink-tan nodule measuring 1.8 cm in greatest dimension, which is not suspicious. The remainder of the parenchyma is dark red and grossly unremarkable.

The isthmus and left thyroid lobe are grossly unremarkable.

The specimen is reviewed by [REDACTED].

Cassette summary: "B1"-"B6" right superior pole nodule, entirely submitted; "B7" and "B8" representative sections of normal thyroid parenchyma ("B7" contains the last small remaining portion of the capsule of the superior pole nodule); "B9"-"B11"

representative sections of mid to lower thyroid nodule; "B12" isthmus; and "B13"- "B10" representative sections of left thyroid lobe (submit 20 total cassettes per [REDACTED]).

C. Received in formalin labeled "right paratracheal level 6 nodes" is a portion of fibroadipose tissue measuring 3.3 x 1.5 x 1.0 cm. The specimen is dissected for lymph nodes, and there are two possible lymph nodes grossly identified, measuring 0.4 to 0.5 cm in greatest dimension.

The nodes are entirely submitted in cassette "C1." The remaining fibroadipose tissue is entirely submitted in cassettes "C2" and "C3."

Microscopic Description

Microscopic examination has been performed on all slides. The pathologic diagnosis encompasses the essential microscopic findings of this case.

Interpretation performed at [REDACTED]
[REDACTED]

[REDACTED]